

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<answer>C. Fluoxetine</answer>

<think>Reasoning process :

1. The patient's symptoms such as lack of attention span , inattention , restlessness , and impulsivity are characteristic of Attention Deficit Hyperactivity Disorder (ADHD).
2. Given the severity of symptoms and the presence of significant attention problems as reported by the mother , the child has likely been diagnosed with ADHD .
3. The family history of ADHD and major depressive disorder in the mother also increases the risk of ADHD in the child .
4. Since the child is already exhibiting behavioral problems and has a history of disruptive behavior , the physician needs to address the ADHD symptoms .
5. Fluoxetine is a commonly used medication for treating ADHD , especially in children . It belongs to the Selective Serotonin Reuptake Inhibitor (SSRI) class of medications , which is often used to treat ADHD .

other possibilities such as B. Methimazole might be considered if there was a family history of thyroid disorders , as it is used to treat Graves ' disease , but there is no indication of thyroid problems . A. Behavior therapy may be useful for behavioral problems , but may not address the underlying ADHD symptoms . D. Hearing aids are not indicated as there is no evidence of hearing loss .